

The patient was found to carry a pathogenic mutation in BRCA1: p.V600E, which is commonly associated with oncogenic signaling.

A second variant was identified in TP53: p.R175H. This variant is frequently observed in multiple tumor types.

Whole exome sequencing revealed a coding DNA change c.1799T>A in BRAF, resulting in an amino acid substitution.

Another clinically relevant variant was detected: c.35delG in the GJB2 gene, which is associated with hearing loss.

We also observed an insertion mutation c.123_124insA in a cancer-related gene.

The genomic coordinate variant g.140453136A>T was reported in chromosome 7.

The SNP rs121913529 is known to affect kinase activity in BRAF.

A second SNP rs334 is associated with sickle cell disease.

The notation p. V600 E (with spaces) was also detected in the literature and should still be captured if normalization is correct.

A malformed variant appears as p.V600-E and c.1799 T > A which tests robustness of parsing.

Another noisy example includes rs 123456 (with a space) and rs-78910 which should ideally be handled.

In clinical notes, we also see references like BRCA2 mutation and EGFR activating mutation without explicit HGVS formatting.

The study used Memorial Sloan Kettering Cancer Center sequencing pipeline for analysis.

Dose escalation study included mutations in KRAS p.G12D and NRAS p.Q61K.

A final edge case: c. 248 G > A should still be recognized if preprocessing is correct.